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Patient Name	: Mr.RAVINDER YADAV	Collected	: 02/Aug/2025 10:37AM
Age/Gender	: 33 Y 1 M 15 D/M	Received	: 02/Aug/2025 12:23PM
UHID/MR No	: CAOP.0000006448	Reported	: 02/Aug/2025 03:00PM
Visit ID	: CAOPOPV08449	Status	: Final Report
Ref Doctor	: Self	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 10010160		

DEPARTMENT OF HAEMATOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*				
HAEMOGLOBIN	14.4	g/dL	13-17	Spectrophotometer
PCV	43.50	%	40-50	Electronic pulse & Calculation
RBC COUNT	5.04	Million/cu.mm	4.5-5.5	Electrical Impedence
MCV	86.2	fL	83-101	Calculated
MCH	28.5	pg	27-32	Calculated
MCHC	33.1	g/dL	31.5-34.5	Calculated
R.D.W	12	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	5,870	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	49.9	%	40-80	Flow cytometry
LYMPHOCYTES	**46**	%	20-40	Flow cytometry
EOSINOPHILS	1.3	%	1-6	Flow cytometry
MONOCYTES	2.6	%	2-10	Flow cytometry
BASOPHILS	0.2	%	0-2	Flow cytometry
CORRECTED TLC	5,870	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	2929.13	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	2700.2	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	76.31	Cells/cu.mm	20-500	Calculated
MONOCYTES	**152.62**	Cells/cu.mm	200-1000	Calculated
BASOPHILS	11.74	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	1.08		0.78- 3.53	Calculated
PLATELET COUNT	155000	cells/cu.mm	150000-410000	Electrical impedence
MPV	12.8	Fl	8.1-13.9	Calculated

Dr.Shivangi Chauhan
M.B.B.S,M.D(Pathology)
Consultant Pathologist

This test has been performed at Apollo Health and Lifestyle Ltd - RRL ASHOK NAGAR

SIN No:AOP250800211

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DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , *NAF PLASMA*	90	mg/dl	74-106	GOD, POD

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.

2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

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DEPARTMENT OF BIOCHEMISTRY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , *WHOLE BLOOD EDTA*				
HBA1C, GLYCATED HEMOGLOBIN	4.7	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	88	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4
DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
2. Trends in HbA1C values is a better indicator of Glycemic control than a single test.
3. Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
5. In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
   A: HbF >25%
   B: Homozygous Hemoglobinopathy.
   (Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)

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DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , *SERUM*				
TOTAL CHOLESTEROL	**204**	mg/dl	0-200	CHOD
TRIGLYCERIDES	115	mg/dl	0-150	GPO, Trinder
HDL CHOLESTEROL	**62**	mg/dL	40-60	CHOD
NON-HDL CHOLESTEROL	**141**	mg/dL	<130	Calculated
LDL CHOLESTEROL	**118.09**	mg/dL	<100	Calculated
VLDL CHOLESTEROL	23.08	mg/dL	<30	Calculated
CHOL / HDL RATIO	3.26		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	< 0.01		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: **Low risk** – No known risk factor of cardiovascular disease.**Moderate risk** – Any one risk factor eg;smoking/hypertension/diabetes mellitus etc.
High risk – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years,history of heterozygous familial hypercholesterolemia. **Very high risk** – clinical evidence of coronary artery disease, long-standing diabetes mellitus existingfor >20 years, etc. **Extremely high risk** – recurrent vascular events.
1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.
2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

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DEPARTMENT OF BIOCHEMISTRY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) , *SERUM*				
BILIRUBIN, TOTAL	**2.36**	mg/dl	0-1.2	NBD
BILIRUBIN CONJUGATED (DIRECT)	**0.69**	mg/dl	0-0.2	Diazotized sulfanilic acid
BILIRUBIN (INDIRECT)	**1.67**	mg/dL	0.0-1.1	Calculated
ALANINE AMINOTRANSFERASE (ALT/SGPT)	21.04	U/l	0-45	IFCC
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	24.9	U/l	0-35	IFCC
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	**1.2**		<1.15	Calculated
ALKALINE PHOSPHATASE	88.58	U/l	53-128	IFCC (AMP buffer)
PROTEIN, TOTAL	8.03	g/dl	6.4-8.3	Biuret
ALBUMIN	4.61	g/dl	3.5-5.2	Bromcresol Green
GLOBULIN	3.42	g/dL	2.0-3.5	Calculated
A/G RATIO	1.34		0.9-2.0	Calculated

Please correlate clinically

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2.Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

Dr.Shivangi Chauhan
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Consultant Pathologist

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RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , *SERUM*				
CREATININE	0.74	mg/dL	0.67-1.17	Enzymatic colorimetric
.eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	121.09	mL/min/1.73m²	>60	CKD-EPI FORMULA
UREA	13.70	mg/dl	13-43	Urease, UV
BLOOD UREA NITROGEN	**6.4**	mg/dL	8.0 - 23.0	Calculated
URIC ACID	5.94	mg/dL	3.5-7.2	Uricase
CALCIUM	9.36	mg/dl	8.6-10.3	Arsenazo III
PHOSPHORUS, INORGANIC	3.42	mg/dl	2.7-4.5	Molybdate
SODIUM	143	mmol/L	135-145	Direct ISE
POTASSIUM	4.6	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	106	mmol/L	98 - 107	Direct ISE
PROTEIN, TOTAL	8.03	g/dl	6.4-8.3	Biuret
ALBUMIN	4.61	g/dL	3.5 - 5	Bromocresol Green
GLOBULIN	3.42	g/dL	2.0-3.5	Calculated
A/G RATIO	1.34		0.9-2.0	Calculated

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Test Name	Result	Unit	Bio. Ref. Interval	Method
HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN) , *SERUM*	0.70	mg/L	0-3	Particle Enhanced Immunoturbidimetric Assay

Comment:

CRP is an acute-phase protein and is useful in the detection and evaluation of infection, tissue injury, and inflammatory disorders. CRP concentrations increase rapidly and dramatically (100-fold or more) in response to tissue injury or inflammation. High-sensitivity CRP (hs-CRP) is more precise than standard CRP and enables a measure of chronic inflammation.Atherosclerosis is an inflammatory disease and hs-CRP has been endorsed by multiple guidelines as a biomarker of atherosclerotic cardiovascular disease risk.

Recommendation by the U.S. Centers for Disease Control and Prevention, and the American Heart Association for assessment of risk for cardiovascular disease in adults-

CARDIOVASCULAR DISEASE RISK	hs-CRP (mg/L)
Low	< 1.0
Average	1.0 – 3.0
High	> 3.0 – 10.0
Possible another source of inflammation/infection	> 10.0

This hs-CRP assay should be used as a means to assess risk of cardiovascular disease or events. A different CRP test (CRP / C-Reactive Protein, Serum) should be used to monitor or assess other inflammatory disorders.

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Consultant Pathologist

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Age/Gender	: 33 Y 1 M 15 D/M	Received	: 02/Aug/2025 12:23PM
UHID/MR No	: CAOP.0000006448	Reported	: 02/Aug/2025 03:03PM
Visit ID	: CAOPOPV08449	Status	: Final Report
Ref Doctor	: Self	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 10010160		

DEPARTMENT OF IMMUNOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , *SERUM*				
TRI-IODOTHYRONINE (T3, TOTAL)	127.754	ng/dL	80-190	CLIA
THYROXINE (T4, TOTAL)	10.145	µg/dL	5-13	CLIA
TSH (Ultrasensitive/4thGen)	2.125	µIU/mL	0.35-4.75	CLIA

Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) and its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active.
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Conditions
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Treatment.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma

Dr.Shivangi Chauhan
M.B.B.S,M.D(Pathology)
Consultant Pathologist

This test has been performed at Apollo Health and Lifestyle Ltd - RRL ASHOK NAGAR
SIN No:AOP250800212

Apollo Health and Lifestyle Limited (CIN - U85110TG2000PLC115819)
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APOLLO CLINICS NETWORK
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Patient Name	: Mr.RAVINDER YADAV	Collected	: 02/Aug/2025 10:37AM
Age/Gender	: 33 Y 1 M 15 D/M	Received	: 02/Aug/2025 04:17PM
UHID/MR No	: CAOP.0000006448	Reported	: 02/Aug/2025 05:01PM
Visit ID	: CAOPOPV08449	Status	: Final Report
Ref Doctor	: Self	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 10010160		

DEPARTMENT OF CLINICAL PATHOLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , *URINE*				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Physical Measurement
pH	7.00		5-7.5	Double Indicator
SP. GRAVITY	1.005		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error Of Indicator
GLUCOSE	NEGATIVE		NEGATIVE	Glucose Oxidase
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Azo Coupling Reaction
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium Nitro Prusside
UROBILINOGEN	Normal		NORMAL	Modifed Ehrlich Reaction
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	2-3	/hpf	0-5	Microscopy
EPITHELIAL CELLS	1-2	/hpf	<10	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
CASTS	ABSENT		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy
OTHERS	ABSENT			Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

Dr.Shivangi Chauhan
M.B.B.S,M.D(Pathology)
Consultant Pathologist

This test has been performed at Apollo Health and Lifestyle Ltd - RRL ASHOK NAGAR
SIN No:AOP250800213

Apollo Health and Lifestyle Limited (CIN - U85110TG2000PLC115819)
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Patient Name	: Mr.RAVINDER YADAV	Collected	: 02/Aug/2025 10:37AM
Age/Gender	: 33 Y 1 M 15 D/M	Received	: 02/Aug/2025 12:23PM
UHID/MR No	: CAOP.0000006448	Reported	: 02/Aug/2025 06:24PM
Visit ID	: CAOPOPV08449	Status	: Final Report
Ref Doctor	: Self	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 10010160		

DEPARTMENT OF SEROLOGY

CHECKMED - IIFB - AHC - PAN INDIA - FY2526

Test Name	Result	Unit	Bio. Ref. Interval	Method
VDRL (RPR) , *SERUM*	NON REACTIVE		NON-REACTIVE	Slide Flocculation

Comment:

Veneral Disease Research Laboratory (VDRL) test is a non-specific screening test and reactive results must be correlated with supportive clinical, historical and epidemiological evidence to arrive at a final diagnosis. Titers of 1:8 and above are considered significant. False positives may be seen in a variety of acute and chronic conditions. It is advisable to confirm diagnosis by TPHA, FTA-Abs, TPI test.

Biological False Positive Results:

(A) Infectious:

1. Bacterial: Scarlet fever, Pneumococcal pneumonia, Bacterial endocarditis, Leprosy, Tuberculosis.
2. Protozoal: Malaria, Trypanosomiasis.
3. Viral: HIV, Measles, Mumps, Viral hepatitis, Chicken Pox, Infectious mononucleosis, etc.

(B) Non-Infectious:

Pregnancy, Advanced cancer, Multiple blood transfusions, Multiple myeloma, Connective tissue disease.

Biological False Negative Results:

May be due to prozone phenomenon. The test must be repeated with higher dilutions of serum, if clinically indicated.

Comparison of RPR, TPHA tests

Test	Use	Limitations
RPR	Screening test. Follow-up titres for monitoring response to treatment.	Biological false positives possible. Repeat testing should be done with a specific test like TPHAfor low titres such as 1:1, 1:2 and 1:4.
TPHA	More specific than RPR	Less sensitive in early syphilis.

RPR : Rising titres are found in active disease and levels subside after successful treatment.

RPR is a simplified alternative to the conventional VDRL test. It has modified VDRL antigen which contains carbon particles. Due to ease of visualisation, the test is sensitive and the test of choice for serum samples.

***** End Of Report *****

Dr.Shivangi Chauhan
M.B.B.S,M.D(Pathology)
Consultant Pathologist

This test has been performed at Apollo Health and Lifestyle Ltd - RRL ASHOK NAGAR

SIN No:AOP250800208

Apollo Health and Lifestyle Limited (CIN - U85110TG2000PLC115819)
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Patient Name	: Mr.RAVINDER YADAV	Collected	: 02/Aug/2025 10:37AM
Age/Gender	: 33 Y 1 M 15 D/M	Received	: 02/Aug/2025 12:23PM
UHID/MR No	: CAOP.0000006448	Reported	: 02/Aug/2025 06:24PM
Visit ID	: CAOPOPV08449	Status	: Final Report
Ref Doctor	: Self	Sponsor Name	: CHECKMED PRIVATE LIMITED
Emp/Auth/TPA ID	: 10010160		

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6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only

Dr.Shivangi Chauhan
M.B.B.S,M.D(Pathology)
Consultant Pathologist

This test has been performed at Apollo Health and Lifestyle Ltd - RRL ASHOK NAGAR
SIN No:AOP250800208

Patient Name	: Mr. Ravinder yadav	Age	: 33Yrs 1Mths 16Days
UHID	: CAOP.0000006448	OP Visit No.	: CAOPOPV08449
Printed On	: 02-08-2025 09:36 AM	Advised/Pres Doctor	: --
Department	: Radiology	Qualification	: --
Referred By	: Self	Registration No.	: --
Employeer Id	: 10010160		

DEPARTMENT OF RADIOLOGY

ULTRASOUND WHOLE ABDOMEN

Liver is normal in size(13.7cm) and shows diffuse increase in echotexture with suggestive of Grade I fatty infiltration. No focal lesion seen in the liver. Intrahepatic bile ducts and portal radicals are normal in caliber.

Gall bladder is partially contracted, **and lumen shows a calculus of size measuriing~18mm with impacted echogenic sludge with** normal **GB wall**.
CBD is not dilated.
Portal vein is normal in caliber.

Both kidneys are of normal size (RK 9.8x5.4cm, LK 9.5x5.5cm), shape and echo pattern. No growth or hydro nephrotic changes seen in either kidney. The parenchymal thickness is normal & cortico-medullary differentiation is well maintained.

Spleen is normal in size (11.3cm) and echotexture.
Pancreas visualized part appears normal.

No free fluid seen in the peritoneal cavity.

Urinary bladder is adequate and shows no mural or intraluminal pathology.
Prostate is normal in Size, vol~26cc **with median lobe intending into bladder base.**

IMPRESSSION: - Cholelithiasis
Grade I fatty liver

Please correlate clinically.

---End Of The Report---

Dr.SEEMA PRAJAPATI
MBBS MD Radio
DMC111002
Radiology

Patient Name	: Mr. Ravinder yadav	Age	: 33Yrs 1Mths 16Days
UHID	: CAOP.0000006448	OP Visit No.	: CAOPOPV08449
Printed On	: 02-08-2025 06:59 PM	Advised/Pres Doctor	: --
Department	: Cardiology	Qualification	: --
Reffered By	: Self	Registration No.	: --
Employeer Id	: 10010160		

DEPARTMENT OF CARDIOLOGY

Transthoracic Echo-Doppler Report

M-Mode/2-D Description:

Left Ventricle: It is Normal. No RWMA. LVEF 60 %.
Left Atrium: It is normal.
Right Atrium: It is normal.
Right Ventricle: It is normal.
Aortic Valve: It is Normal.
Mitral Valve: it is Normal.
Tricuspid Valve: It appears normal.
Pulmonary Valve: It appears normal.
Main Pulmonary artery & its branches: Appear normal.
Pericardium: There is no pericardial effusion.
Inter Atrial Septum: It is intact.
Inter Ventricular Septum: It is intact.

Measurements (mm):

	Observed Values		Normal Values
Aortic root diameter	32 mm		20-36(22mm/M2)
Left Atrial size	32 mm		19-40
	End Diastole	**End Systole**	
Left Ventricle size	47 mm	30 mm	(Ed=37-56)
Interventricular Septum	10 mm	13 mm	(Ed=6-12)
Posterior Wall Thickness	09 mm	12 mm	(Ed=5-10)
L V Ejection Fraction (%)	60 %		55-80%

Doppler Velocities (cm/sec)

Pulmonary valve		**NORMAL**	**Aortic valve**		
Max velocity	0.87		**Max velocity**	1.25	
			Mean velocity		
			Max PG	10	
			Mean PG		
Mitral valve			**Tricuspid valve**		
E	0.78	**Max PG=**	**Max velocity**		
A	0.64	**Max Velocity=**	**TAPSE (>1.5)**	2.0	
E'		**Mean PG=**	**E/E' (<6)**		
E/E'		**Mean Velocity=**			

Regurgitation

	MR		**TR**
Severity	Trivial	**Severity**	Nil
Max velocity		**RVSP**	
	AR		**PR**
Severity	Nil	**Severity**	Nil

Final Interpretation:

No RWMA. Global LVEF- 60 %.
Normal cardiac chambers. Trivial MR, No AR, No TR.
Normal LV Systolic function. Normal diastolic function.
No intra-cardiac clot/vegetation/pericardial pathology.
IVC is normal with normal respiratory variation.

This report is not for any medico-legal purpose.

---End Of The Report---

Dr.RAJNI SHARMA

MBBS MD DM Cardiologist
DMC 22672
Cardiology

Patient Name	: Mr. Ravinder yadav	Age	: 33Yrs 1Mths 16Days
UHID	: CAOP.0000006448	OP Visit No.	: CAOPOPV08449
Printed On	: 02-08-2025 06:58 PM	Advised/Pres Doctor	: --
Department	: Cardiology	Qualification	: --
Reffered By	: Self	Registration No.	: --
Employeer Id	: 10010160		

DEPARTMENT OF CARDIOLOGY

CARDIOLOGY
CARDIAC STRESS TEST – (TMT)

TREADMILL TEST REPORT
Medication: None

Protocol: BRUCE

	Resting	Peak exercise	Recovery 3 (min)	4	6	8
HR/min	73	162	107			
B.P. mm Hg	117/74	135/95	111/88			

Reason for termination

THR Achieved

Events during exercise and recovery

ECG Changes: Baseline ECG - NORMAL

Symptoms : **None**

Arrhythmia : **None**

TET: 10:30mins **METS: 13.40.** **MHR (% THR): 86%**

Impression

TMT IS NEGATIVE FOR REVERSIBLE MYOCARDIAL ISCHEMIA.

Appropriate chronotropic & BP response.

Good Exercise Capacity.

---End Of The Report---

Dr.RAJNI SHARMA
MBBS MD DM Cardiologist
DMC 22672
Cardiology

Patient Name	: Mr. Ravinder yadav	Age	: 33Yrs 1Mths 16Days
UHID	: CAOP.0000006448	OP Visit No.	: CAOPOPV08449
Printed On	: 02-08-2025 06:59 PM	Advised/Pres Doctor	: --
Department	: Cardiology	Qualification	: --
Reffered By	: Self	Registration No.	: --
Employeer Id	: 10010160		

DEPARTMENT OF CARDIOLOGY

ECG

Observation :-

1. Normal Sinus Rhythm.
2. Heart rate is 60 beats per minutes.
3. No pathological Q wave or ST-T changes seen.
4. Normal P,QRS,T waves and axis.
5. No evidence of chamber, hypertrophy or enlargement seen.

Impression:

WITHIN NORMAL LIMITS.

---End Of The Report---

Dr.RAJNI SHARMA
MBBS MD DM Cardiologist
DMC 22672
Cardiology